DELEGATION PRE-READ

Asia-Pacific Digital Health Interoperability Bootcamp 2026

Sydney, Australia | 18–20 October 2026 | AWS Office, Sydney

Co-convened by CSIRO Strengthening Standards Capability Project (SSCP), the World Bank and the Pacific Community (SPC), with support from the Australian Government, Department of Foreign Affairs and Trade (DFAT).

Why read this
Two pages on why the bootcamp exists, the ideas it builds on, and what to bring so your country leaves with a clear, measurable action.

1 WHY THIS BOOTCAMP, WHY NOW

Awareness of digital health standards and interoperability is growing across Asia and the Pacific, but many countries are still early in implementation. The hard step is turning training, pilots and regional commitments into **country-owned architectures, investment plans, procurement requirements and working systems**.

Sydney bootcamp is the **regional checkpoint**. Countries test their progress against peers, refine early outputs, and line up national work with the global experts and standards bodies in the room such as, HL7 International, SNOMED International, openEHR, Regenstrief and WHO.

The gap we are closing. Some of the main challenges we face are fragmentation and overlap, driven by the proliferation of new digital tools built **without common standards**, or **lacking the support of a shared vision** – *Dr Tedros Ghebreyesus at the launch of the Global Initiative on Digital Health, 20 February 2024*

The bootcamp's answer: check each country's plan against its peers and the standards bodies, before it becomes an investment.

2 WHERE BOOTCAMP SITS ON THE BROADER REGIONAL PATHWAY

YOU ARE HERE

PHASE 1 · 2024–25
Awareness & Foundations
Regional training, workshops and sandbox learning on FHIR, terminology and interoperability.

PHASE 2 · APR 2026
Co-Design & Alignment
Shared artefacts (Common Dataset, FHIR IG concepts); Blueprint readiness.

PHASE 3 · OCT 2026, SYDNEY
Validation & Learning Exchange
Review progress, learn across regions, align with global standards bodies.

PHASE 4 · 2026–28
Implementation & Scaling
Technical assistance, national adoption, institutionalisation.

3 THE BOOTCAMP AT A GLANCE

Day 0 Sun 18 Oct	**Closed multi-stakeholder workshops (invitation only).**
Day 1 Mon 19 Oct	**Connected Care: the International Patient Summary (IPS).** Regional priorities; alignment with WHO and global trends. Knowledge support from standards bodies for the shared patient summaries (HL7, openEHR, SNOMED International, Regenstrief), a cross-border referral use case, and a workshop shaping a **Pan-Asia-Pacific IPS Connectathon**.
Day 2 Tue 20 Oct	**Digital Public Infrastructure for Health (DPI-H).** The WHO DPI-H reference architecture and the World Bank Blueprint Toolkit. Country journeys from Australia, Fiji, the Philippines, Indonesia, and Mongolia; Australian DPI master class; trust frameworks for cross-border sharing; **country Design Lab**; Regional Action Plan.
Days 3–5 21–23 Oct	**SNOMED CT Expo.** Registered participants continue with the global terminology community. **PHIT Digital Teams**. Technical side meeting with UNICEF/ SPC/WB **(invitation only)**

4 WHAT YOUR COUNTRY LEAVES WITH

IPS
A regional patient-summary plan
A grounded understanding of patient summaries, validated regional IPS direction and a proposed pathway to a IPS Connectathon.

DPI-H
A country capability pathway
Prioritise shared registries and services that many programmes reuse, and avoid wasting resources on duplicated, siloed systems.

Community
Regional network that lasts
One national action that can start within 12 months, and one shared regional action, feeding a Regional DPI-H Action Plan.

5 YOUR ROLE IN THE ROOM

Ministry of Health leaders
Ask: Which use case comes first, who governs it, and how does it fit our national Blueprint?

Clinician champions
Ask: Does the patient summary hold what a clinician needs at referral or handover? Clinical needs come first, technical limits second.

Technical focal points
Ask: What is our baseline (FHIR, SNOMED CT, LOINC, registries), and where are the gaps?

Finance & ICT ministries
Ask: What can we build once and reuse across programmes, and how do we make conformance a condition of procurement?

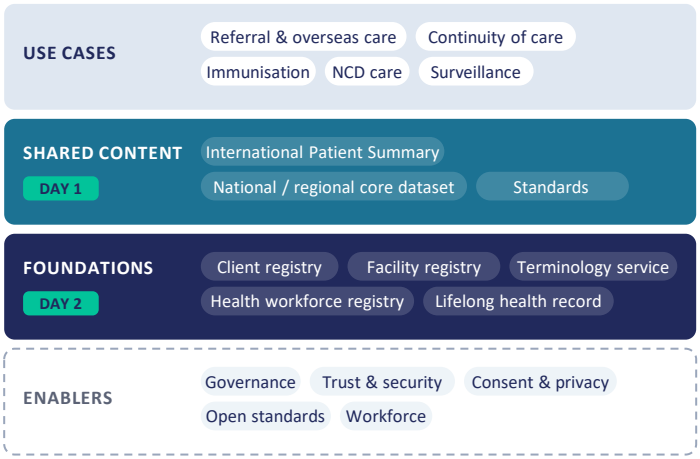
WORLD BANK GROUP · Pacific Community Communauté du Pacifique · Australian Aid · CSIRO

Why peer validation matters. A national action is only realistic when it is measured against what comparable countries have actually delivered. 10+ Pacific and Southeast Asian countries will test their approaches against each other in Sydney.

Scope note. The bootcamp sets enabling environment, national direction and readiness. Detailed technical builds (HIE architecture, master patient index design, deployment patterns) belongs to a later phase.

6 TWO THREADS, ONE INFRASTRUCTURE

The IPS is the **clinical content that travels** with the patient. DPI-H is the **shared infrastructure it travels on**. DPI-H means digital resources built as a shared means for many health functions, not tools for one programme, and it extends national identity, payment and data-exchange infrastructure rather than duplicating it.



A lifelong record depends on the client, facility and workforce registries to attribute records and establish provenance. That is why these registries form the essential core. **Build once, reuse everywhere**.

7 REGIONAL SIGNALS TO LEARN FROM

Fiji Moving from fragmented systems to "One System" through governance, FHIR, core registries and phased delivery. Leads the Pacific Common Clinical Dataset (PCDI) and the Fiji Core FHIR IG.

Philippines Every partner mapped its work against OpenHIE at national roundtable, endorsed by DOH, exposing duplication and silos. National SNOMED CT licence, PH Core FHIR IG, and technical sandbox.

Indonesia SATUSEHAT includes a regulatory sandbox to test digital health applications and rules before launch.

India ABDM built shared registries for people, facilities and professionals, plus a per-transaction incentive scheme to drive adoption.

Australia Its national core dataset (AUCDI) feeds the AU Patient Summary, which conforms to the IPS: a national path to an international standard.

8 KEY TERMS

FHIR HL7 standard for structuring and exchanging health data.

LOINC Codes for laboratory tests and clinical observations.

openEHR Vendor-neutral, reusable clinical information models.

FHIR Implementation Guide (IG) Rules for how FHIR is used in a given context.

DPI-H Shared, reusable, publicly governed digital health foundations.

SNOMED CT Clinical terminology that gives each concept one shared meaning.

IPS A minimal set of essential patient information that can travel across borders.

CDI / PCDI The agreed national or Pacific minimum clinical dataset.

Connectathon A time-boxed event where systems are tested against a standard.

Blueprint A country plan that sequences digital health investment.

9 FROM GLOBAL STANDARD TO NATIONAL PRACTICE

International standard	Core dataset (what)	National Core IG (how)	Use-case profiles
HL7 FHIR, IPS, SNOMED CT, LOINC. Deliberately general so every country can adopt them.	The minimum clinical data a country or region agrees to share, adapted from IPS to local priorities.	Fixes national identifiers, terminologies and mandatory elements, and carries government authority.	Adapt the core further for a specific workflow and are tested with vendors before scale-up.
Owned by the standards bodies	e.g., PCDI (Pacific), AUCDI (Australia)	e.g., Fiji Core, PH Core, Pacific-Core	e.g., eReferral, immunization

Adopt, adapt and reuse international content wherever possible and localise only where national practice genuinely differs.

10 GLOBAL PATTERNS: WHAT WORKS

Govern first, specify second 01

Secure clinical consensus, country ownership and governance before detailed technical specification. Do not add governance afterwards.

Assess before you invest 02

Start with a readiness assessment and a Blueprint that sequences governance, registries and interoperability. Do not finance digital health investment before an assessment.

No country starts from zero 03

Identify your starting point. Integrate before you replace, replace at natural transition points, and make architectural conformance a condition of procurement.

Test before you trust 04

Adopting a standard on expert opinion alone creates costly legacies. Prove it in sandboxes and connectathons with real systems first.

One accountable Role 05

Appoint a national digital Technical Working Group (TWG) with authority to say no. Program incentives otherwise favour vertical systems over shared infrastructure.

Community sustains capability 06

Learn (training of trainers), **Build** (shared sandboxes), **Connect** (communities of practice that outlast funding and reduce reliance on consultants).

11 TRAVEL REQUIREMENTS: YOU MUST

- [ ] **Choose one priority use case**, for example referral, continuity of care, immunisation or NCD care.
- [ ] **Map what already exists:** registries (client, facility, workforce), terminology licences, standards in use, and the status of your strategy or Blueprint.
- [ ] **Know who decides:** the governance body, technical advisory group and architecture lead.
- [ ] **Bring current-state work:** core dataset drafts, FHIR IG work, Blueprint notes from the Fiji or Manila bootcamps.
- [ ] **List 2–3 critical gaps or dependencies** for your use case.
- [ ] **Agree a draft national and regional action** as a delegation, ready to test in the Design Lab.